



Memorial Sloan Kettering
Cancer Center

Submitted electronically via Regulations.gov

February 19, 2026

Assistant Secretary for Technology Policy and
The Office of the Nation Coordinator for Health Information Technology
Department of Health and Human Services
Mary E. Switzer Building
Mail Stop: 7033A, 330 C Street SW
Washington, DC 20201

Re: "HHS Health Sector AI RFI"

To Whom It May Concern:

On behalf of the Artificial Intelligence (AI) Governance Committee (AIGC) at Memorial Sloan Kettering Cancer Center (MSK), we appreciate the opportunity to provide commentary on steps that the U.S. Department of Health and Human Services (HHS) can take to "establish a forward-leaning, industry-supportive, and secure approach to accelerate the adoption and use of AI as part of clinical care."

As a specialty cancer hospital and National Cancer Institute (NCI)-designated comprehensive cancer center, MSK takes pride in delivering cutting edge, high-quality cancer care to our patients. In 2024, we cared for more than 74,000 patients, including 26,000 inpatient hospital admissions and more than one million hospital outpatient visits.

AI tools have exciting potential to enhance clinical care in a wide variety of ways. MSK has already deployed several U.S. Food and Drug Administration (FDA)-approved AI systems into clinical practice, and our clinicians, scientists, and research faculty are also actively developing new AI clinical tools and systems. To support responsible use of AI, the AIGC at MSK oversees the registration, review, and approval of all clinical AI tools at our institution. MSK's AIGC also ensures that each system undergoes thorough evaluation before deployment, and remains subject to continuous oversight throughout its life cycle.

To accelerate the adoption of AI in clinical settings, we encourage HHS to address barriers to AI development and implementation arising from vendor policies that assert restrictive foreground intellectual property rights. Restrictive intellectual property and data usage clauses are a significant administrative hurdle and are frequently found in standard contracts with Electronic Medical Record (EMR) and Picture Archiving and Communication System (PACS) companies. Although health systems legally own patient data, vendors frequently argue that AI

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models developed from this data (housed and extracted from their platforms) constitute foreground IP that they then own.

This creates a chilling effect on innovation. Internal efforts within hospital systems are discouraged from developing novel AI models, since ownership cannot be guaranteed due to these restrictive contract conditions. In turn, clinicians and patients lose out on advancing patient care, removing those closest to the everyday issues in healthcare from the development process. Further, this creates a dependency on these vendors, as many offer their own versions of AI models that require continued subscription to the vendor and/or incur additional costs. Health care systems then effectively pay twice – once for the initial subscription, and second for the AI model derived partly from their own data on that vendor's platform.

Even if these barriers are overcome, these restrictions can prevent deployment of these AI models with other vendors, as interoperability is not guaranteed. This creates vendor-dependent ecosystems, effectively siloing progress depending on which health care systems have existing contracts with which companies.

HHS needs to dismantle these contractual barriers to foster an environment of provider ownership that ensures exponential growth of AI in healthcare. Without this, development will remain tethered to the interests of outside vendors acting to ensure a noncompetitive (and ultimately stagnant) future.

In addition, the MSK AIGC reiterates our feedback in response to a previous FDA RFI (FDA-2025-N-4203) on evaluating AI-enabled medical device performance. Confidence in the accuracy and reliability of AI-enabled medical devices is crucial to widespread adoption in clinical practice, and we urge HHS to address the following areas through policymaking and oversight:

Data curation

Establishing the necessary pipelines for ongoing monitoring of AI tools has proven to be highly burdensome, with data curation posing an especially significant challenge. As a case example, consider our hospital's implementation of an FDA-approved AI tool to aid in screening mammography interpretation. The tool generates a suspicion score for each mammogram, which is available to radiologists during image interpretation. Safe and responsible AI use requires the ongoing monitoring of standard performance metrics (e.g., sensitivity, specificity, etc.) over time. However, calculating these metrics across large numbers of exams requires aggregating the AI outputs and merging them with ground truth labels and demographic information from the electronic health record, which in our experience has been a challenging task. The vendor-generated AI outputs are stored within each patient's exam jacket in PACS. The vendor provides no built-in support to aggregate these scores into a usable format. As a



result, the simple task of generating lists of AI outputs linked to patient IDs has proven to be a resource-intensive process.

To address these challenges, we call on the FDA to require that AI output scores be stored in a standardized format—analogue to the DICOM standard for imaging data or the HL7/FHIR standard for electronic health records—to facilitate streamlined data extraction. We also strongly recommend that vendors be required to provide software that can integrate with electronic health record data, thereby enabling customers to monitor system performance and usage over time.

Data analysis

We encourage the FDA to support the development of third-party registries capable of receiving curated data from clinical sites. These registries would provide an option for centers that wish to offload the analytical work of ongoing monitoring, and they would also enable dissemination of performance metrics across institutions, offering valuable insights to the broader clinical community.

Vendor feedback tools

Additionally, we wish to highlight a practice by some AI companies of requesting user feedback using model monitoring as a pretense, while the feedback serves a different purpose – to provide the company with data that can be used to validate newer version models and collect labeled data for retraining their proprietary weights without remuneration. This turns clinicians into unpaid annotators, helping the parent company's internal model development rather than providing oversight. We believe that such feedback tools should be redesigned to only provide feedback to the health systems licensing the product. With this shift, healthcare providers will be encouraged to use quality assurance functions without fear of their clinical judgments being monetized or repurposed for commercial model training.

Short term performance metrics

For AI models designed to make longer term predictions (e.g., 5-year cancer risk), we recommend that vendors be required to establish performance metrics on a shorter time horizon (e.g., 1 year), so that potential problems or concerning trends can be addressed without undue delay.

Reimbursement

Who pays for AI and its oversight? Lack of clinical and technical funding is a barrier to widespread adoption and responsible use. The Centers for Medicare and Medicaid Services (CMS) should support payments for clinician training and utilization, AI governance, infrastructure, and continuous monitoring without allocating from the budget-neutral Medicare Physician Fee Schedule, which would then require decreased reimbursement for other physician services.



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In conclusion, we believe that the elimination of restrictive intellectual property policies by vendors is critical to accelerating the development and adoption of AI as part of clinical care. We also believe that safe and effective AI governance is a critical underpinning to adoption of AI in clinical settings, and can be achieved through empowering healthcare institutions and clinicians with readily available, model-provided tools to monitor the known risks of deployed AI systems. Thank you for your consideration of our feedback.

Sincerely,

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